

APA GUIDELINES on Measurement-Based Care

APA TASK FORCE ON PROFESSIONAL PRACTICE GUIDELINES FOR MEASUREMENT-BASED CARE

APPROVED BY APA COUNCIL OF REPRESENTATIVES
FEBRUARY 2025



**AMERICAN
PSYCHOLOGICAL
ASSOCIATION**



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PROFESSIONAL PRACTICE GUIDELINES FOR MEASUREMENT-BASED CARE

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APA Task Force on Professional Practice Guidelines (PPGs) for Measurement-Based Care (MBC)

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Introduction

Primary Purpose of Guidelines

The purpose of the American Psychological Association (APA) Professional Practice Guidelines (PPGs) for Measurement-Based Care (MBC) is to educate psychologists on competencies for conducting MBC. Also referred to in the literature with names such as routine outcome monitoring and feedback-informed treatment, MBC is a clinical process in which data are used to monitor treatment progress and processes to enhance communication and inform treatment planning during the provision of psychological intervention (Boswell et al., 2023). MBC is thought to improve patient outcomes via multiple proposed mechanisms, including making clinicians aware of discrepancies between their own case conceptualization and what the assessment data indicate, signaling when treatment adjustments are needed, enhancing patients' understanding of their problems, and increasing patient engagement and the therapy alliance (de Jong et al., 2021). MBC can be conceptualized as three phases. Psychologists *collect* data via the repeated administration of measures throughout treatment, *share* and discuss the data with patients, and *act* by using the data in collaborative conversations about how treatment is going and to support decisions around treatment planning and goal setting (Barber & Resnick, 2023).

MBC is an evidence-based practice that has been shown to improve symptom outcomes and reduce treatment dropout across a range of clinical conditions, service settings, and age groups (de Jong et al., 2021). Some research suggests that MBC is particularly beneficial for patients who are "not on track" for positive outcomes and that its effects may be increased using clinical support tools to help clinicians know how to take action in response to feedback (Barkham & Mellor-Clark., 2003). Because of this research support, MBC is included as an essential component of APA's definition of evidence-based psychological practice in health care (APA, 2021c). MBC holds potential to increase health equity by reducing barriers to engagement and supporting personalized treatment for patients representing diverse backgrounds and identities (Barber et al., 2024; Connors et al., 2023). Merely administering measures to monitor treatment is not synonymous with high-quality MBC (Wampold & Miller, 2023). MBC is most effective when psychologists integrate it into the process of psychological treatment, routinely administering and tailoring measures to the patient, and use MBC to build transparent collaboration and therapeutic alliance with patients. These PPGs focus

specifically on clinical knowledge and skills that psychologists can learn and apply to competently deliver high-quality MBC.

Scope

These guidelines aim to educate psychologists on a broad range of clinical competencies that make up MBC for application in the course of psychological treatment. These include: explaining the purposes and processes of MBC to patients and other treatment participants; selecting measures; administering measures; using MBC data in discussions with patients and to support treatment-planning; delivering MBC in a way that is inclusive and empowers patients representing the full range of personal identities; and understanding ethical and legal considerations. These guidelines are not specific to any particular type of psychological intervention. Therefore, they are applicable regardless of therapeutic orientation and can be integrated into the vast majority of evidence-based psychological interventions and treatment settings. Whenever feasible, the authors integrated existing research literature into their recommendations. However, given the lack of experimental studies comparing specific approaches to MBC, specific recommendations regarding practical application are often guided by theory and informed by evidence.

There are several topics that fall beyond the scope of this PPG. MBC utilizes data from brief assessment measures, but this PPG is not intended to cover the use of psychological instruments and tests for formal psychological assessment or diagnosis, as APA has a published guideline dedicated to this topic (APA, 2020). Although organizational and contextual factors are crucial in any clinical practice, including MBC, this PPG will not address these complexities, nor is it a guide on implementation strategies (see Dollar et al., 2020, for an example of an implementation planning process for MBC). There is a critical need for additional research to guide the field on how best to support implementation of MBC to inform future guidelines on this issue (Lewis et al., 2019; Rognstad et al., 2023). Finally, although MBC data can also aid psychologists in conducting clinic-level quality improvement efforts and/or be used in value-based payment models to increase reimbursement rates, the authors determined these uses to be outside the scope of these guidelines. MBC data can be explored across all patients to evaluate the effectiveness of treatment, assess the impact of introducing new practices on patient outcomes, and/or identify clinic-level needs for additional training and improved implementation of clinical practices (e.g., Connors et al., 2021). Dollar and colleagues (2020) include this use of data in their implementation planning process as well. Aggregated MBC data can also support psychologists in demonstrating the value and effectiveness of treatment to payers and policymakers, which can have downstream impacts on reimbursement policies and care costs (Fortney et al., 2017; Rothrock et al., 2024).

Need for Guidelines

Professional practice guidelines can aid in the uptake of evidence-based practices based on the current state of the literature and on professional consensus (APA, 2015b). Boswell and colleagues (2023) proposed a PPG on measurement-based care to enhance public benefit (i.e., improved service delivery, engagement with underserved populations), provide professional guidance, and address legal and regulatory issues with MBC. While data-driven clinical decision making has been explicitly encouraged in clinical care since the late 1990s (Howard et al., 1996), adoption of MBC has been relatively low in the field, as reported in a number of samples (Keepers et al., 2023; Lewis et al., 2019). Many factors likely contribute to this sobering trend, including barriers at the patient, provider, and/or system levels. These real or perceived barriers, which may include concerns about confidentiality, utility, cost, or competing demands, need to be addressed to realize optimal use of MBC (Bilimoria et al., 2014; Bugatti et al., 2023). Further, providers have expressed resistance and fears that MBC will interfere with their autonomy to practice or be used to evaluate their performance (Bobbitt et al., 2012). There has yet to be a consensus on MBC best practices, which is needed to ensure MBC is truly evidence-based and poised for successful implementation, in part by addressing some of these barriers (Wampold & Miller, 2023).

A central principle of these guidelines is that MBC is conducted within an evidence-based practice framework that incorporates research evidence, patient preferences and culture, and clinical judgment, and we endeavored to provide guidance about how to conduct this practice when the research may be underdeveloped (e.g., when measures have not been validated for a particular patient population). Therefore, these guidelines aim to clarify the core practices for using MBC in clinical care with patients, which will likely be most useful in addressing client- and clinician-level barriers to MBC implementation. However, we acknowledge that an even broader, multi-level, systemic approach will be necessary to address all perceived barriers to successful MBC implementation, in turn potentially improving health at the population level (Bilimoria et al., 2014; Bugatti et al., 2023). It is our hope that a clear guideline on the clinical application of MBC can set the stage for this work.

Purpose of Guidelines

The intended audience for the guidelines presented here is psychologists and other healthcare professionals who may be delivering behavioral healthcare services to youth and adult patients. The guidelines can also be useful to individuals providing education and training on MBC to psychologists and other healthcare professionals (e.g., Cooper et al., 2021). The guidelines provide specific recommendations for the use of MBC in the context of evidence-based psychological

practice. Psychologists and other healthcare professionals may consult the existing guidelines on evidence-based psychological practice in healthcare (APA, 2021c) and psychological assessment and evaluation (APA, 2020) and the Standards for Educational and Psychological Testing (American Educational Research Association, American Psychological Association, & National Council on Measurement in Education, 2014) for additional recommendations. MBC can be applied to both youth and adults across a range of treatment settings and modalities (e.g., individual and group therapy). The presented guidelines include recommendations for MBC use with patients representing diverse identities, including “social identities that correspond to societal differences in power and privilege,” including “race, ethnicity, culture, gender, gender identity and expression, sexual orientation, socioeconomic status, religion, spirituality, disability, age, national origin, immigration status, and language,” (APA, 2023, p. 4). Psychologists may consult other published APA guidelines, including those on working with boys and men (APA, 2018a), girls and women (APA, 2018b), transgender and gender nonconforming people (APA, 2015a), sexual minority persons (APA, 2021b), military service members and veterans (APA, 2021a), persons with disabilities (APA, 2022), older adults (APA, 2024), persons with low income and economic marginalization (APA, 2019), and persons with multicultural identity and intersectionality (APA, 2017b) for additional guidance when using MBC with patients with diverse social identity factors.

Distinction Between APA Standards and Guidelines

The term *guideline* refers to statements that suggest or recommend specific professional behavior, endeavor, or conduct for psychologists. Guidelines differ from standards. Standards are mandatory and, thus, may be accompanied by an enforcement mechanism; guidelines are not mandatory, definitive, or exhaustive, and expressly not intended to be used in legal proceedings or enforcement actions. Guidelines are aspirational in intent and are not intended to guide regulatory action or establish a standard of care. They aim to facilitate the continued systematic development of the profession and to promote a high level of professional practice by psychologists. A particular set of guidelines may not apply to every professional and clinical situation within the scope of that set of guidelines. As a result, guidelines are not intended to take precedence over the professional judgments of psychologists that are based on the scientific and professional knowledge of the field (see APA, 2017a, Std. 2.04; APA, 2015b).

Compatibility with APA Ethics Code

These guidelines are informed by relevant and authoritative APA standards, guidelines, and positions including the *Ethical Principles of Psychologists and Code of Conduct* (APA, 2017a), the *APA Record Keeping Guidelines* (APA, 2007), the *Guidance for Developers and Users of Professional Practice Guidelines* (APA, 2015b), the *APA Guidelines for Psychological Assessment and Evaluation* (APA, 2020), and the *APA Guidelines on Evidence-Based Psychological Practice in Health Care* (APA, 2021c). Furthermore, the assumptions and principles that guide the *APA Multicultural Guidelines: An Ecological Approach to Context, Identity, and Intersectionality* (APA, 2017b) are infused throughout the guidelines. Thus, these guidelines are aligned with professional standards and evidence-based practices to offer the best guidance for MBC. Guideline 6 speaks to specific legal and ethical considerations related to MBC. Psychologists are aware of the standards of practice for the settings in which they practice, and they are expected to comply with those standards; these guidelines are not intended to supersede any federal and state laws and regulations.

Background

In 2023, Boswell and colleagues highlighted the need for MBC guidelines, building upon the work of the APA’s Advisory Committee for MBC and the Mental and Behavioral Health Registry. In response, the Board of Professional Affairs (BPA) appointed an eight-person working group of subject matter experts to draft these guidelines, including one liaison representing BPA.¹ The MBC workgroup represented a diverse range of interests and expertise characteristic of the profession of psychology, including knowledge of MBC adoption and implementation, evidence-based assessment, and health equity and population health. The selection process included the dissemination and completion of a diversity matrix by each workgroup candidate to ensure a wide range of expertise, settings, and experience was represented. A completed draft of the guidelines was circulated for Board and Committee review, along with a 60-day public comment period in accordance with Association Rules 30-8. The subsequent review by APA boards, committees, ethnic psychological associations, students and early career psychologists, other professions, and communities of interest was intended to ensure that diverse perspectives and feedback were received and incorporated in the final version of the guideline (APA, 2015b). None of the workgroup members or their affiliated organizations reported financial interests that could influence the review, approval, or implementation process. The guideline developers did not receive external financial support for this project. No funding was received to help prepare these

1 In 2023, the APA appointed the Measurement-Based Care Workgroup dedicated to developing a professional practice guideline: Amanda Jensen-Doss, PhD (Chair); Jessica Barber, PhD; Lee Cooper, PhD; Russell DuBois, PhD; Alyssa Gatto, PhD; Trina Orimoto, PhD; and Kelli Scott, PhD. APA Board of Professional Affairs member Zeeshan Butt, PhD, acted as a liaison to the working group. APA Staff Members C. Vaile Wright, PhD, Leanna Fortunato, PhD, and Aaron Jones, MA, provided direct support to the working group.

guidelines, hold meetings, or conduct the literature review. No funds, grants, or other support was received for this project's development other than what was allocated to support APA Boards and Committees to meet and develop guidance. The guidelines developers complied with APA's policy on conflict of interest.

The guidelines were informed by decades of empirical research, theory, and policy related to MBC, assessment, testing, risks, and competencies, building off the original draft guideline by Boswell and colleagues (2023) and subsequent commentaries on it (Bugatti et al., 2023; Wampold & Miller, 2023). Although the literature review was extensive, it was not exhaustive. However, all workgroup members have considerable experience studying and applying MBC in diverse professional settings and represent diversity in age, sex, ethnicity, career stage, etc., as assessed by the aforementioned diversity matrix. Thus, workgroup members made every effort to consider and integrate broad-based professional consensus and alternative views (e.g., Wampold & Miller, 2023) in both the development and review of guidelines.

Status and Expiration Date

This document is set to expire 10 years from the date of approval by APA Council of Representatives. After this date, users are encouraged to contact the APA Practice to confirm that this document remains in effect.

Definitions

Several terms related to the use of MBC and its impacts are used throughout these guidelines. The term **patient** refers to the individual, group member, couple, or family receiving psychological services. While the patient is the focus of services, MBC often involves gathering and sharing assessment data with other important informants, such as family members, teachers, or other significant individuals in the patient's life. For concision, the term patient often includes these individuals, such as when Guideline 1 refers to "presenting a rationale for using MBC" to patients; any informants involved in completing MBC are often involved in other steps of MBC, such as discussing MBC data.

Throughout the guidelines, the term **measure** refers to the tool used to assess treatment processes and outcomes. This can include standardized rating scales or idiographic, or individualized, measures tailored to patients. The results from these measures are referred to as **MBC data**, and this can include raw scores from the measures or interpreted feedback such as plots of scores over time or indicators of whether scores fall above or below clinical cutoffs. The term **clinical encounter** is used to refer to an occasion where services are provided, such as an individual or group psychotherapy, or case management session.

Finally, the guidelines refer to some treatment process factors, or nonspecific factors thought to be related to client outcomes. The term **therapy alliance** is used in a comprehensive manner to refer both to the bond between the patient and therapy and to agreement on the tasks and goals of treatment (Elvins & Green, 2008), as well as the alliance between group members, which is known as cohesion (Burlingame, Fuhrman, & Johnson, 2002). **Engagement** is also used broadly to refer to the multiple aspects of participating in services, such as attending clinical encounters, participating in the tasks of treatment, completing treatment homework, and successfully completing a full course of treatment (Becker et al., 2018).

The Guidelines

GUIDELINE 1

Psychologists are encouraged to explain to patients and any other treatment participants the purpose of MBC and how data will be stored and used.

Rationale

Psychologists use assessment for different purposes (e.g., cognitive assessment, psychodiagnostic assessment), therefore, they strive to provide an explanation of the purposes of assessment as part of initiating MBC with patients and other treatment participants, such as parents. This is important for several reasons. MBC is a collaborative process that actively engages patients in providing and interpreting assessment data (Boswell et al., 2023; Duncan & Reese, 2015). Collaboration in MBC starts with presenting a rationale for using MBC to patients (Barber & Resnick, 2023) to build patient trust and foster collaboration in the MBC process. Though there may be variability, many patients will also wish to have a clear understanding of how the MBC data will be used (Laver et al., 2023). When patients fully understand the purpose and process of MBC and can appreciate how it will affect their treatment, they are best able to fully collaborate in the process (Zimmerman & McGlinchey, 2008) and report benefits such as: productive conversations with their clinician on progress, priorities, and how they might like treatment to change; increased ability to focus their thoughts on what they would like to discuss in an encounter; and increased self-reflection and awareness of their symptoms and/or functioning over time (Solstad, 2019). One mechanism hypothesized to account for MBC's positive impact on outcomes is an improved therapy

alliance (de Jong et al., 2021), which can be conceptualized as consisting of both the bond between the patient and therapist (or between the patient and other patients in a group setting) and agreement on the tasks and goals of treatment (Elvins & Green, 2008). A collaborative relationship grounded in MBC may facilitate both aspects of the alliance. Such a relationship begins with patients clearly understanding the MBC process.

In contrast, a lack of understanding of MBC and its purposes can affect how patients respond to MBC measures and therefore reduce the benefits of MBC. Patients' answers are based on their interpretation of the items, their own experiences, and how they make meaning. Patients self-appraise, make sense of their experiences, determine how those align with the measure questions, and learn about terms with which they may be unfamiliar (Truijens, 2021). By providing a complete rationale for MBC, psychologists can orient and engage patients in this process and reduce the potential for bias in how the patients respond to the items (e.g., over or underestimating symptoms or distress). This discussion sets the stage for ongoing collaborative discussions throughout treatment. The orientation process can also alleviate concerns patients might have about MBC. For example, they may worry that their psychologists will not look at completed measures or will use them in ways that will not benefit treatment (Borjesson & Bostrom, 2020; Solstad et al., 2019; Talib et al., 2018).

Expectations impact treatment effectiveness (Greenberg et al., 2006) and lack of progress might demoralize some (Errazuriz & Zilcha-Mano, 2018). Therefore, it is also important that the rationale for MBC normalizes the need for treatment adjustments while also empowering patients to use MBC data

discussions as an opportunity to express their own concerns.

Application

Early in psychological care, psychologists strive to orient patients to what MBC is and why it is used in psychological interventions. They also strive to clearly communicate the processes or logistics of how MBC is carried out. These two areas of focus are presented separately below for ease of discussion. We encourage all information in the next two sections to be discussed with patients. However, psychologists are empowered to integrate and order the information in whatever way works best for them and their patients.

Psychologists strive to educate patients about what MBC is and why it is used in care. Psychologists aim to emphasize the importance of multiple sources of information (e.g., clinical interview data, behavioral observations, collateral reports from teachers, family, etc.) for treatment monitoring (APA, 2020). Key in this discussion is how MBC data add value when combined with the psychologist's own clinical judgement and the information patients share spontaneously without any prompting. Psychologists strive to empower patients to think about how measures can support self-reflection on their experiences and highlight that MBC provides an additional way to communicate important information to their psychologist. Psychologists help patients understand how conversations about measure scores and their interpretation provide another opportunity for increased self-awareness and insight. If patients struggle to understand why repeated assessment is a part of the process (see Guideline 3), it can be helpful for psychologists to normalize for patients that, while it is anticipated that treatments will help

them and result in meaningful improvements, this may take time, progress may not be linear, and there may be setbacks, making ongoing monitoring with MBC an important part of the mental health treatment process. Psychologists may choose to liken MBC to the standard practice of repeated assessments used in other areas of health care, such as monitoring blood pressure. Furthermore, psychologists strive to actively encourage patients to ask questions and share any concerns they may have as part of information sharing and obtaining consent. Patients may benefit from repeated discussions of this information as they acclimate to MBC (Brooks Holliday et al., 2021).

Psychologists strive to clearly communicate the processes or logistics of how MBC is carried out. Patient experience of MBC is most positive when measures apply to their perceived needs (Laver et al., 2023). Psychologists therefore strive to explain why specific measures might be chosen by linking patient-identified needs or goals to specific items on applicable measures and/or by including patients in decisions of what to monitor and what measures to use. Psychologists endeavor to ensure patients understand what questions and terms mean and to clarify any misunderstandings that arise. Psychologists aim to explain how and when patients will complete measures and how and when the psychologist will review them. Options for administering measures for MBC include pencil and paper, office- or clinic-housed technology (e.g., kiosks or tablets), and Health Insurance Portability and Accountability Act (HIPAA)-compliant computer-based or mobile apps. Psychologists strive to ensure patients are comfortable with the format being used and to align with patient preferences where appropriate when options are available. Patients may also benefit from training on how to use devices if these will be employed; therefore, psychologists may assess patient comfort level with technology and direct them to resources as available.

Psychologists strive to inform patients about who will have access to the data and where data are stored. Further, psychologists aim to explain any potential benefits, risks, and/or threats to confidentiality when discussing how data are collected and stored.

Discussions of expectations also include information on how often measures will be collected and when psychologists will view the data. If asynchronous technologies are used to collect MBC data outside of a live encounter (e.g., if patients complete measures on personal devices prior to a visit), psychologists aim to communicate when they will receive and review patient responses. This is particularly important if a crisis arises. To ensure patient safety, psychologists strive to provide information to patients about ways to reach out in a crisis to them and/or to other resources (e.g., the National Suicide Hotline, 988, or local emergency services). Finally, as is the case with all mental health treatment and assessment, MBC is voluntary, and though psychologists strive to help patients understand that MBC is a best practice that will enhance their care, patients have the right to refuse to participate. Psychologists may offer the option for MBC at a later time as patients may see benefits as they increasingly engage in psychological treatment, but if patients continue to refuse, psychologists strive to support that decision by either modifying their approach or referring to a provider who does not use MBC.

MBC includes sharing and discussing data with patients (see Guideline 4); therefore, psychologists aim to explain when and how that will be done. They may remind patients that these discussions will provide them with a chance to ask questions and discuss the measures and any reflections or additional information the patient may wish to provide. When multiple family members and/or treatment team members are involved in care, this includes discussions about who will see the data. In a group setting, the psychologist may set the expectation that data

discussion may involve providing themes from the data rather than sharing individualized data due to the confidential nature of an individual member's data or asking members directly about their experience in the group and how it maps on to their assessment data.

Finally, as detailed in Guideline 6, there are a number of legal and ethical issues associated with use of MBC, including obtaining informed consent, storing data securely, and following HIPAA and other privacy policies.

GUIDELINE 2

Psychologists strive to select evidence-based measures that can feasibly be used to monitor the outcomes and processes most relevant to patient care.

Rationale

Selecting measures is an important step in ensuring that MBC is grounded in clinically useful data that can aid clinical decision-making (Garza et al., 2024). To best select measures, psychologists strive to be guided by empirical evidence supporting their psychometric properties, as well as appropriateness, feasibility, acceptability, and effectiveness in facilitating treatment outcomes and improving patient care. Prior to or upon beginning MBC, it is recommended that psychologists give careful and deliberate thought to the selection of evidence-based measures, what to measure, and who will complete measures (de Jong et al., 2023). While standardized measures are useful in facilitating normative comparisons to support clinical decision making, it can be useful to supplement them with idiographic or individualized measures (Jensen-Doss et al., 2018). For example, at the start of care patients might create a list of the problems most distressing to

them or their personal resilience factors then rate each item every week on a scale from 0-4 (Weisz et al., 2011). Idiographic measures can be used to monitor changes in symptoms or goals framed in patients' own words and to help ensure that MBC is measuring changes in the problems or goals most important to patients. Psychologists strive to include their patients in the measure selection process as much as possible to facilitate patient buy-in and to ensure that the measures cover domains that are relevant to the patient's care (Lewis et al., 2019).

Application

Because MBC is grounded in assessment, psychologists' professional competencies in assessment are highly relevant to selecting measures for MBC. The *APA Guidelines for Psychological Assessment and Evaluation* (APA, 2020) provide guidance related to assessment, and the guidelines related to the selection of tests are particularly useful for psychologists to reference at the measure selection stage of MBC. Those guidelines highlight that psychologists strive to choose assessment tools with sound psychometric properties that are appropriate for the evaluation purpose, population, setting, and context (APA, 2020, Guideline 6), aim to use multiple sources of relevant and reliable information when possible and appropriate (APA, 2020, Guideline 7), and strive to be aware of the need for assessment to involve appropriate normative comparisons as indicated (APA, 2020, Guideline 8). Similarly, the *Standards for Educational and Psychological Testing* (American Educational Research Association, American Psychological Association, & National Council on Measurement in Education, 2014) provide a comprehensive resource for psychologists to use to understand psychometric factors and other important issues related to measure selection and interpretation.

Psychometric considerations. Score reliability (consistency of measurement), validity (accuracy of measurement), and ability to detect change have been identified as the key psychometric properties for standardized patient-reported outcome measures (Frost et al., 2007). Because MBC measures are designed to be administered repeatedly over short periods of time (see Guideline 3), test-retest reliability (i.e., stability of scores over time) may not be as relevant for these measures, although having high reliability within community samples where outcomes are not anticipated to change rapidly can indicate that changes on the measure during treatment are reliable. Strong internal consistency for multi-item measures supports the notion that all items are measuring the same thing. Cutoffs for "good" reliability for MBC measures may be higher than for measures being used for other purposes. That is because they are being used to make comparisons in scores for an individual over time, which can be more error prone. Thus, reliability cutoffs around 0.9 are recommended (Frost et al., 2007). In terms of validity, content validity (i.e., do the measure items cover relevant content) and construct validity (i.e., does the measure converge with other similar measures and diverge from dissimilar measures) are important to ensure that the measures are accurately measuring the domains of interest. Finally, ideally, MBC measures have evidence that they can detect change over time. While this is often not reported in the psychometric data for measures, evidence that measures have detected changes in prior studies or in clinical settings can raise the psychologist's confidence that the measure will be able to assess changes occurring during treatment. The absence of such data, however, does not preclude the use of the measure for MBC. Because idiographic measures are tailored to the patient's specific goals or concerns, some research suggests they may be more sensitive to change than standardized measures (Lindhiem et al., 2016).

Practical considerations. In addition to these psychometric factors, there are practical considerations relevant to selecting MBC measures (Lewis et al., 2019). This includes access to measures, mode of administration, administration time, and ease of scoring, technology, training, and consultation. Brief measures that are easy to administer, score, and interpret facilitate integration into routine clinical workflows. The term "brief" is commonly used to describe the recommended length of a measure or battery of measures in MBC due to the often-cited patient barrier of response burden (Scott & Lewis, 2015). The recommended time to complete measures can vary depending on the specific measure being used, treatment goals, and context of care. In the context of a direct practice setting, we propose psychologists strive to routinely administer measures that take the patient no more than 2-10 minutes to complete. Further, and aligned with our recommendation below of what domains to measure, this could readily include a reliable and valid measure of symptomology, well-being, and treatment process, each of which can be fewer than 10 items; idiographic measures can also be used to shorten the assessment. For specific longer measures or a larger battery of measures, it is recommended to adjust the frequency of measures (e.g., every other week or monthly) so as not to overburden the client on any single occasion. It is recommended that psychologists select measures that are anchored in a period appropriate for the measurement schedule. For example, a measure that asks about symptoms over the past 6 months would not be sensitive to changes between clinical encounters. Finally, to promote accurate assessment and reduce bias, psychologists strive to ensure that measures are appropriate for the patient and balance assessing both symptoms and strengths-based recovery domains; Guideline 5 discusses strategies for tailoring MBC selection based on patients' unique identities and needs.

Cost is another practical consideration. Costs can be minimized by using free measures and simple strategies for administering them, such as paper and pencil. Many measures have constraints on their use. Some may be used for free in certain circumstances, such as for the provision of direct clinical care, while others require payment of licensing fees. Nonetheless, many free measures are available (see e.g., Beidas et al., 2015 and Becker-Haimes et al., 2019). Use of idiographic measures can also decrease cost, as these are created specifically for the patient. Technologies that facilitate MBC further add to the cost burden, but it is important to emphasize that technological solutions are not required to do high-quality MBC. In virtual care, psychologists may send measures to patients to have on hand, which patients can complete and share back via secure means of communication, including simply reading their responses to the psychologist.

Selecting what to measure. Identifying what to measure is crucial for effective MBC implementation (Scott & Lewis, 2015). A guiding principle is that MBC measures are relevant to clinical care and generate actionable feedback that supports clinical decision-making. Further, MBC is based on a collaborative approach to measure selection and data review, which can include identifying measures that assess psychological domains and functions meaningful to the patient and/or group (Barber & Resnick, 2023). Specific to identifying what to measure, it is recommended that, at minimum, psychologists strive to use a combination of a targeted outcome measure; a psychological functioning or subject well-being measure that captures important aspects of daily living; and/or a treatment process measure that can help the clinician identify when key processes, such as the therapy alliance or the patients' use of skills between sessions, are not operating as desired. It is understood that the preferred domain to assess and the actual measure(s) administered will be based, in some part, on the orienta-

tion of the clinician and the patient goals. When indicated, measures of risk and harm are important to include alongside measurement of other relevant outcomes. In general, the more information a health service provider has, the better they can track progress and process, and thus personalize the treatment plan. Being able to monitor multiple elements (symptoms, functioning, and alliance) could promote treatment engagement and secure better overall outcomes. Standardized MBC measures can be very useful to facilitating comparison of patient scores and score trajectories to established norms and monitoring for things such as emerging symptoms, whereas individualized measures can be useful to assess outcomes or processes that are highly specific to the patient or that are not covered in published standardized measures. Therefore, using a combination of standardized and individualized measures can be very useful. Below are broad domains to consider for monitoring with MBC:

Clinical outcomes. This includes measures that directly assess the symptoms or outcomes relevant to the patient's presenting problem(s), diagnosis(es), and treatment goal(s). For example, if treating depression, then using measures that assess symptoms of depression would be appropriate.

Psychological functioning/quality of life. This includes measures that also evaluate broader aspects of functioning and well-being that are not tied to a single presenting condition. Such measures aim to assess well-being, psychological functioning, relationships, family, social activities, work or school activities, etc. Regardless of whether a person is experiencing a psychological disorder, how they are functioning may provide a valuable index of how they are coping with life in general and may often overlap with specific goals

the psychologist and patient are working on.

Treatment process. This includes measures that capture and track important therapy processes such as therapy alliance, clients' use of or confidence in using treatment skills, treatment satisfaction, or treatment adherence.

Informant considerations. Decades of research indicate that different informants often provide different descriptions of a patient's symptoms (De Los Reyes et al., 2015). Rather than being viewed as error to be disregarded, these informant discrepancies are increasingly being conceptualized as meaningful reflections of clinically relevant information such as variability in behavior across contexts; therefore, different informants' perspectives on the client can be valuable to inform case conceptualization and treatment planning (De Los Reyes et al., 2022). As such, psychologists strive to gather MBC data from multiple relevant informants, when possible and when the patient agrees this may be informative, either using parallel versions of the same measures or administering different measures when appropriate (e.g., having parents complete a measure of caregiver strain). While administering measures to multiple informants may be particularly useful in youth mental health treatment (Jensen-Doss et al., 2020), use of multi-informant MBC has also been found to be effective in adult treatment such as couple therapy (e.g., Anker et al., 2009).

Normative comparisons. Some MBC measure developers have used normative data to create feedback alerts to support clinical decision making. Some of these are based on identifying whether a patient's current score is above or below a clinical cutoff and whether changes being observed over time represent reliable change versus random fluctuations. Having information about the clinical cutoff for a measure and the reliable change index

(RCI, that is, the amount of change on a measure that exceeds what is considered random) can be used to identify patients who are unchanged (change has not exceeded the RCI), improved (change has exceeded the RCI but the current score is still in the clinical range), recovered (change exceeds the RCI and the current score is not in the clinical range), or deteriorated (change exceeds the RCI, but not in the desired direction; Jacobson & Truax, 1991). Some MBC systems have built upon these indices to create empirically based metrics for characterizing the change that would be expected over time if patients are on track for positive treatment outcomes and ways to alert clinicians if their clients fall outside these expected change trajectories; some approaches further tailor these trajectories based on client characteristics (Lutz et al., 2022). Psychologists aim to select MBC measures based on normative data drawn from a sample of individuals who are similar to the patient on relevant dimensions, such as age, gender, race/ethnicity, diagnosis, etc., and interpret data cautiously in situations where this is not possible.

GUIDELINE 3

Psychologists aim to administer and score measures at regular and repeated time points throughout care.

Rationale

Prevailing definitions of MBC rest on the premise that psychologists aim to use measure results actively during a course of care, not just at the outset (e.g., psychological testing or diagnostic assessment) or retrospectively after care has ended (e.g., as in a case evaluation). The process of MBC as described in these guidelines is dependent on the availability of multiple measure results for the review of patient

progress. In this way, the repeated nature of administration represents a necessary component of MBC and a distinguishing factor from ancillary and secondary uses of clinical measurement such as program evaluation, quality assurance, or care coordination.

The emphasis on repeated and regular assessment in MBC is informed by growing recognition in psychological treatment that, to detect and address problems and challenges as early as possible, it is necessary to take frequent and ongoing measurements of outcomes, psychological functioning, and the treatment process. Referring to examples in physical health, an individual at a higher risk of acquiring a disease (because of family history, for instance) will increase their chances of early detection and successful treatment when frequent tests are done. Similarly, in psychotherapy settings, regular and systematic measure administration and scoring improves the clinician's ability to detect the worsening of symptoms and important process factors such as rifts in the treatment alliance (e.g., Lambert, 2010), counteracting their tendency to overestimate their own abilities and treatment benefits and underestimate worsening or stagnation (e.g., Chow et al., 2024; Walfish et al., 2012; Whipple & Lambert, 2011). More frequent viewing of MBC data is associated with better outcomes (Bickman et al., 2011), as it enables clinicians to tailor treatments more effectively.

Application

Psychologists strive to administer and score measures regularly and repeatedly throughout a course of care and in such a way that allows for the clinical features of care delivery (i.e., clinical impressions and case conceptualizations) to be actively informed by measure results. When determining the frequency of measure administration for a patient, psychologists aim to consider multiple factors including (but not limited to) the patient population,

the type of treatment being provided, the expected length of treatment, the availability of measure resources, the recommended frequency of administration of the measure (based on the psychometric studies of the measure developers and others), the sensitivity to change of the construct being measured, and the patient's comfort with measure completion. While acknowledging these factors, psychologists endeavor to select a measure administration frequency that (a) is no less than the time frame specified in the instructions for a given measure, (b) is greater than measure administrations at the beginning and end of care only, and/or (c) represents a time frame in which the constructs being measured are anticipated to be sensitive to change. While not all chosen measures must be administered at the same cadence, incorporating outcome measurements at every outpatient psychotherapy encounter can help make MBC a habit and facilitate its integration into ongoing clinical decision-making. In treatment setting types such as inpatient treatment or intensive day treatment, it is not feasible or clinically appropriate to administer measures at every encounter, but in these settings psychologists strive to do so frequently enough that the data are useful for monitoring progress and can guide treatment planning and goal setting. In settings with team-based models of care, it is appropriate for one member of the team to administer measures that all members of the team can then incorporate into their respective treatment with the patient.

Administration method. Assessment administration may occur via self-report paper and pencil forms presented to the patient during a clinical encounter or in a waiting room, via verbal administration by the psychologist during a clinical encounter, or digitally using remote assessment technology. When determining the appropriate method for administration for a given patient or population, it is important to consider factors such as patient privacy, patient

safety (e.g., a patient report of risk to self or others on a remote assessment elicits a greater level of risk and requires more proactive crisis management resources compared with a same-patient report conducted in a clinic waiting room), access to technology (e.g., the completion of remote assessments in digital format may not be feasible among patients who live in areas with low network connectivity), demand characteristics of the environment (e.g., thoughtful, honest responses might be unlikely if clients are hurried or distracted), and availability of resources (e.g., certain assessments may not be available in digital formats or may be prohibitive due to cost).

Administration timing. To obtain the most current understanding of patient progress, it is typically recommended to administer patient-reported outcome measures shortly before (i.e., within 24 hours) or during the initial portion of a clinical encounter. It is recommended that patient-reported outcome measures be administered either during the first part of a clinical visit or within 24 hours before to capture the most accurate assessment of the patient's current status. Collecting these measures before or at the start of the encounter might prevent the scores from being influenced by the therapeutic benefit of the clinical interaction, which could mask the patient's true presenting condition. On the other hand, assessments of therapy alliance are best suited for administration toward the end of a clinical encounter to elicit feedback on the encounter itself, which can be addressed prior to an encounter ending (Duncan, 2012). Care needs to be taken when administering assessments remotely in between clinical encounters to reduce the risk of patients with a high-risk assessment response having to wait a long time until they have contact with a provider. If urgent needs are communicated to the psychologist during any part of the MBC process, psychologists aim to use clinical judgment in responding, including abstaining from or ceasing MBC until

these needs, be they medical or mental health, can be attended to appropriately.

Multi-informant measure administration. Other informants contributing to the patient's MBC might complete measures using the same method and timing as the patient. However, this might not always be the case. For example, teachers might complete a teacher version of a measure that a youth patient might be asked to bring to an upcoming clinical encounter.

GUIDELINE 4

Psychologists strive to collaborate with patients to routinely review and take the data from past and current measures into account when formulating clinical impressions, case conceptualization, and the treatment plan.

Rationale

Routinely reviewing and sharing MBC data from current and past clinical encounters can support psychologists in (a) assessing patient treatment progress in the moment and over time; (b) having collaborative discussions with patients about current symptoms and functioning alongside overall treatment progress; and (c) identifying the need to adjust clinical impressions, case conceptualization, and the treatment plan.

First, routine review of MBC data allows psychologists to evaluate a patient's current symptoms, functioning, and/or other important outcomes over time. MBC data review can complement what is learned about patient progress through clinical observation and discussion during the clinical encounter. Psychologists can use MBC to assess change since the last assessment point as well as since the beginning of treatment, enabling clinicians

to more rapidly and accurately identify patients who are not responding to treatment or are clinically deteriorating (Boswell et al., 2023; De Jong et al., 2021). MBC data review can be used to form and adjust clinical impressions and case conceptualization regardless of a psychologist's theoretical orientation, typical treatment or formulation approach, or client diagnoses (Scott & Lewis, 2015).

Second, MBC data is most effective when it is also shared with the patient to strengthen the therapy alliance, facilitate communication, and enable shared decision making in treatment (Boswell et al., 2023; Scott & Lewis, 2015). The use of data visualizations in the form of graphics, charts, or images can serve as an effective tool for both the provider and the patient in facilitating better understanding and comprehension of the data (Abudiyab & Alanazi, 2022). Collaborative review of MBC feedback also enables patients to share their interpretation of measures based on identity factors (e.g., neurodiversity, cognitive impairment, cultural identity, gender identity, sexual orientation, etc.; Barber et al., 2024; McFayden et al., 2021; Schwartzman et al., 2023). MBC feedback data can aid psychologists in forming and adjusting clinical impressions and in building and revising case conceptualizations or treatment plans. MBC data can aid psychologists in clarifying diagnoses; selecting treatment protocols; testing clinical hypotheses; identifying predisposing, precipitating, perpetuating, and protective factors affecting client symptoms; and considering the need for additional clinical services (e.g., more frequent psychotherapy sessions, referral to housing or employment services; Boswell et al., 2023; Valenstein et al., 2009). Additionally, MBC data can enable psychologists to assess a patient's level of insight and understanding of outcomes over time. Incomplete or inconsistent MBC data may enable psychologists to better understand patient insight and further individualize the MBC approach for patients (e.g., by using an idiographic approach matched to patient needs).

Finally, in collaborative or team-based models of care, MBC data may be valuable in team discussion around patient status and in team treatment planning.

Application

Psychologists strive to review MBC data each time a measure is given and to incorporate MBC data when formulating and clarifying patient diagnoses, testing clinical hypotheses, and assessing patient treatment response by reviewing how MBC data change over the course of treatment. They endeavor to review MBC data when attempting to answer key questions such as whether there is a significant concern, if progress is being made, and if there is a need to change the current case conceptualization.

Psychologists aim to share data with the patient and incorporate the information into treatment. This is facilitated by taking an open and nondefensive stance toward the data. Psychologists strive to share and discuss data with the patient at each clinical encounter (or each time a measure is given). Psychologists can discuss current responses and changes in responses over time to both individual questions and overall measure scores (since the last assessment and since starting treatment). They may use data visualization tools to guide the discussion, but psychologists strive to be aware that the utility of data visualizations is not “one-size-fits-all” and may need to be customized to fit the needs of the patient (Polhemus et al., 2022).

Patients’ unique perspectives on their symptoms and functioning is invaluable, and they can report on alignment or misalignment of MBC data with their lived experience. Therefore, psychologists aim to ask patients if data align with their subjective lived experience and cultural identity factors, use misalignment as an opportunity for a collaborative exploration, and actively encourage patients to elaborate on

anything that they want their psychologist to understand as they interpret the MBC data. This could facilitate new insights for the patient and/or psychologist. Discussions of discrepancies can also inform troubleshooting as to how to adjust MBC to improve alignment (e.g., providing education on misunderstood terms or selecting alternative measures that might better capture the patient’s experience).

Psychologists aim to incorporate MBC data in the development and ongoing evaluation of the treatment plan via collaborative discussion with the patient. When making treatment plan changes, psychologists strive to engage in collaborative decision making with the patient, which may include inviting the patient to share their needs, preferences, and agreement with planned changes (Barber & Resnick, 2023). MBC data indicating clinical improvement may suggest whether the current level of care and psychotherapy approach are appropriate to address presenting symptoms/diagnoses and can be continued or tapered off (i.e., if planning for relapse prevention or discharge). MBC data indicating lack of progress or clinical deterioration can indicate a need to address an emergent concern in the current clinical encounter, consider modification of the treatment plan, and/or address potential problems with treatment plan execution or integrity. A nonexhaustive list of modification examples includes more frequent psychotherapy sessions, a change to the psychotherapy approach, addition of other forms of psychotherapy (e.g., group-based treatment), medication management, case management, referral to a higher level of care (e.g., intensive outpatient, partial hospitalization), or referral to other support services (e.g., housing, employment, educational services).

Multi-informant considerations. In many cases, it is necessary to review MBC data from multiple informants to ensure the data are accurate and informative. This is common practice when working with children, adolescents,

couples, families, neurodiverse, or cognitively disabled patients (De Los Reyes et al., 2022). When including multiple informants in the MBC process, psychologists strive to provide rationale for reviewing multiple sources of data and provide information to each informant about how data will be shared, discussed, and used in treatment. Psychologists also aim to review MBC data from multiple informants to identify both agreement and discrepancies in reporting, normalize that discrepancies are very common, and use this information to promote collaborative discussion about patient progress and treatment planning. Psychologists may view discrepancies in reporting as an opportunity to better understand a patient’s experience in different contexts (e.g., school, work, home), understand differences in perceptions of the patient’s symptoms across informants, and make changes to the case conceptualization or treatment plan to address patient needs across these contexts or address issues such as communication difficulties that could be contributing to diverging perceptions (De Los Reyes et al., 2022).

Imminent risk considerations. Many MBC measures provide information on suicidality, self-harm, or other high-risk behaviors that are necessary to review at each clinical encounter. To ensure the patient is safe, psychologists aim to check these items prior to the end of the encounter to enable additional risk assessment and safety planning, including potential initiation of applicable statutory mandatory reporting requirements. Additionally, consistent and marked increases in MBC scores may indicate that a patient is experiencing or is at risk for clinical deterioration or relapse. Ongoing assessment of risk can also help to track patterns and identify periods where risk is increasing and support clients in the development of safety planning and relapse prevention skills (Rudd et al., 2022).

GUIDELINE 5

Psychologists endeavor to be attentive and responsive to individual characteristics that might impact the delivery of MBC and strive to tailor the MBC process to meet individualized needs of patients.

Rationale

Psychologists recognize that all patients may identify with diverse “social identities that correspond to societal differences in power and privilege, and thus to the marginalization of some groups based on specific attributes—e.g., race, ethnicity, culture, gender, gender identity and expression, sexual orientation, socioeconomic status, religion, spirituality, disability, age, national origin, immigration status, and language” (APA, 2023, p. 4) and that these identities often intersect and have differential impact across contexts. Psychologists appreciate how MBC, as a patient-centered approach, is consistent with a health equity lens. Psychologists appreciate MBC’s potential to improve outcomes, which is particularly important for disadvantaged groups, by signaling the need for and fostering collaborative discussions about treatment adjustments. Though the literature in this area is only emerging, psychologists also endeavor to appreciate MBC’s potential to enhance mental health treatment for diverse groups due to its emphasis on therapeutic alliance, empowerment of patients to be active participants in their care, and improved communication, as well as its ability to help set up realistic expectations for the process of mental health treatment (Barber et al., 2024; Connors et al., 2023).

The APA *Guidelines on Evidence-Based Psychological Practice in Health Care* stipulates that psychologists engage in psychological practice and assessment in a way that is “appropriate for the setting, purpose, and population,” with psychologists endeavoring

to respect patient identities and ecological contexts and adapt the approach accordingly to increase the effectiveness of the psychological intervention (APA, 2021c, p. 10). For this reason, psychologists endeavor to be attentive and responsive to individual characteristics that might affect the delivery of MBC and strive to tailor the MBC process to meet individualized needs of patients.

Application

When collecting MBC measure data, psychologists discuss methods of administration and ensure that patients are comfortable with them. To promote accurate assessment and reduce bias, psychologists strive to ensure that measures are appropriate for the patient; ideally, MBC measures will have psychometric support for use with patients at the patient’s developmental level and from their linguistic and cultural background. Psychologists also aim to select measures that are appropriate to the patient’s level of cognitive functioning in terms of reading level and clarity of response options and to consider options such as simplified rating scales or pictures where appropriate. The selection of which informants complete measures is also undertaken with the consideration of factors that might interfere with the patient’s ability to participate in MBC, such as age or disability status. Selecting measures and informants collaboratively with patients can focus on getting their input on the appropriateness of the measures and the measurement approach to their care. Psychologists aim to select MBC measures with normative data drawn from a sample of individuals who are similar to the patient on relevant dimensions, such as age, gender identity, race/ethnicity, diagnosis, disability status, acculturation, etc. Though some measures have been validated and normed with diverse groups (e.g., the PHQ-9), many have not (Beidas et al., 2015; Molock et al., 2023). Psychologists aim to reflect on this limitation and interpret

data cautiously with patient input in situations where an otherwise clinically appropriate measure lacks validation within a group that represents an important dimension of the patient’s identity. Psychologists recognize that failing to engage patients in MBC because of such shortcomings introduces a disparity in treatment that is best avoided. As an alternative to standardized, validated measures, psychologists may also consider using idiographic measures (see Guideline 2), which are individualized to align with what is most important to individual patients (Jensen-Doss et al., 2018).

Psychologists aim to solicit and incorporate patient preference and input in identifying options for measure completion as appropriate given patient ability across different domains of functioning (e.g., visual impairment, cognitive impairment). If technologies are used as part of MBC workflows, psychologists endeavor to familiarize themselves with ways that introducing technology into clinical workflows may disadvantage some patients (Liu et al., 2019; Sisodia et al., 2021; Wallace et al., 2016) and strive to correct these disadvantages through steps such as providing access to technology in the clinical setting or reverting to other forms of assessment when access to technology is a barrier. Finally, because measures for MBC are text-based, psychologists strive to be aware of barriers to literacy and patients’ levels of English language proficiency that may affect patients’ ability to use measures, and they aim to incorporate patient preference and input in identifying other options for measures completion as appropriate. For example, non-English translations are available for some measures, and although measures are completed by the patient independently, psychologists may consider reading items to patients with low literacy. Further, psychologists endeavor to consider the fit of idiographic measures as an alternative to better reach these patients.

When discussing MBC data, psychologists strive to develop a shared

understanding aligning with the patient's view, which may include discussion of how patients experience using MBC and how they understand the MBC measures and data in the context of their unique intersecting identities, and to approach these discussions with authentic curiosity and cultural humility. Consistent with the *APA Guidelines for Psychological Assessment and Evaluation* (APA, 2020), psychologists do not pathologize behavior or experiences that are normative for a patient based on group identification.

GUIDELINE 6

Psychologists strive to be aware and knowledgeable of ethical, legal, and regulatory considerations regarding MBC.

Rationale

Valuing and conducting ethical behavior and adhering to relevant legal and regulatory parameters in MBC is essential to upholding the highest duty of a professional psychologist: protecting the public. While MBC offers benefits such as improved treatment outcomes and engagement, it also raises ethical, legal, and regulatory considerations across a range of domains such as: consent and assent for MBC; documentation of MBC data; who has access to MBC data; the role of family, caregivers, and/or supportive others in MBC; issues related to data storage and information security; and psychologist competencies related to the administration, scoring, and interpretation of assessments used for MBC. Addressing ethical, legal, and regulatory issues is central to the responsible implementation of MBC, ensuring it benefits both patients and psychologists while upholding principles of privacy, autonomy, and equity.

Application

MBC results in patient-generated data, which becomes part of the records that are maintained by psychologists (APA, 2017a, Part 6). Psychologists strive to ensure that patient information, including MBC data, is adequately protected according to legal requirements such as HIPAA and other applicable privacy laws and regulations. Psychologists aim to inform patients that MBC data will become part of their records and about their or their organization's policy for safeguarding confidential mental health information in electronic or other health records. Psychologists are responsible for supporting the implementation of appropriate security measures to protect patient data from unauthorized access, breaches, or misuse. This includes ensuring that any technology used to collect or score MBC data meets appropriate security standards. Psychologists strive to provide a clear and comprehensive description of their MBC practices, including the purpose, benefits, and potential risks associated with MBC; how the data will be used and shared; and whether that data will be accessed by other treatment team members or third parties for administrative and/or billing purposes. Research with patients suggests they may hold concerns about whether MBC data truly will be used to support their care rather than being used solely for administrative purposes or that the data could even be used to limit their access to care (e.g., Solstad et al., 2019), which underscores the importance of obtaining informed consent and assent to ensure patients understand and agree to participate in MBC. In youth mental health care, this includes a discussion of how MBC data will be shared within the family and with other informants (e.g., school counselors). Psychologists strive for necessary training and expertise to administer and interpret measures accurately. Lack of competence in MBC practices could lead to misguided choice of measures, failure to review data in a timely manner, misinterpretation of data, or inappropriate

treatment recommendations. While MBC can enhance patient engagement in treatment decisions, there is a need to ensure that patients retain autonomy and are actively involved in the decision-making process (Barber & Resnick, 2023). The documentation of MBC, which includes MBC data and discussions of data, progress, etc., with patients in clinical progress notes and treatment plans, facilitates accurate record keeping and allows psychologists and other team members to refer back to this information in the future (Barber & Resnick, 2023). Psychologists aim to evaluate the appropriateness of MBC for each patient on an ongoing basis, considering factors such as patient compliance, treatment progress or lack thereof, and privacy or security issues.

Future Directions

While we hope these initial guidelines prove useful for professionals using MBC, these represent just a first step in clarifying the practice. There are a number of remaining issues related to MBC that could be addressed in future guidelines. First, MBC may face barriers to implementation at the system, psychologist, and patient level across different care settings. A core future direction is to further study and provide guidelines for MBC implementation, including optimal implementation strategies necessary to overcome these barriers and support MBC use at scale. Second, MBC is likely enhanced by an array of clinical decision support tools and emergent digital technologies to guide treatment decision making and planning. Although a review of decision support tools is beyond the scope of this guideline, additional research is needed to identify optimal decision support pathways to aid psychologists in clinical decision making. Relatedly, while significant strides have been made in the development of measurement models to help alert clinicians when action is needed in response to feedback (Lutz et al., 2022), there is a need for further refinement of these models and the development of guidelines to help psychologists understand and use them. Finally, this guideline focuses primarily on the application of MBC in the context of individual psychotherapy. MBC can, although many of the principles (e.g., ethical and cultural considerations) are also be applied applicable to group psychotherapy. Published resources specific to the application of MBC in group psychotherapy, and guidelines regarding how to effectively apply MBC in groups represent a key future direction. are available to the interested reader (MacNair-Semands & Whittingham, 2023).

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